INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Modified FOLFOX-6

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
14 days
See the active treatment phase

Treatment stages
1
Explained in the treatment reference

Regimen source
NCCP 00209
Version 10a

Day 1: oxaliplatin, folinic acid and fluorouracil, followed by a 46-hour fluorouracil pump. A cycle lasts 14 days.

Treatment stages

01 **14-day treatment cycle**
Infusion + home pump

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

Modified FOLFOX-6
National Cancer Control Programme (NCCP) 00209

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. New chest pain, breathing difficulty, severe diarrhoea, inability to drink, reduced urine, uncontrolled bleeding or a rapidly worsening symptom needs prompt assessment.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

Your medicines and what they do

Modified FOLFOX-6 combines oxaliplatin, folinic acid and fluorouracil. It can be used after colon-cancer surgery or to treat advanced colorectal, stomach or gastro-oesophageal-junction cancer. The aim and duration depend on the indication; ask which applies to you.

PLATINUM CHEMOTHERAPY

Oxaliplatin

Eloxatin



Links deoxyribonucleic acid (the cell’s genetic material) (DNA) strands

Oxaliplatin makes links within DNA. These interfere with copying and repair, which can stop a cancer cell dividing. Healthy tissues can also be affected.

Platinum enters cell → DNA links form → Copying is disrupted

What to watch for

Cold can trigger tingling or throat discomfort. Tell the team about persistent numbness, difficulty with buttons or walking, or infusion symptoms. New breathing difficulty always needs assessment.

CHEMOTHERAPY MODULATOR

Folinic acid

Calcium leucovorin



Strengthens fluorouracil action

Folinic acid helps fluorouracil remain bound to an enzyme needed to make DNA. In this regimen it increases fluorouracil activity and can also increase its unwanted effects. It is not a vitamin supplement to start yourself.

Folinic acid → Stronger enzyme binding → Fluorouracil effect increases

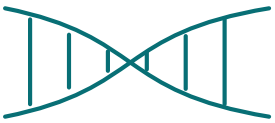
What to watch for

The combined treatment can increase mouth soreness and diarrhoea. Report these promptly. Rare allergic reactions can occur.

CHEMOTHERAPY

Fluorouracil

5-FU



Disrupts cell building blocks

Fluorouracil interferes with the production and use of DNA and RNA, the instructions cells need to grow. The infusion extends exposure over approximately two days.

Drug enters cell → DNA / RNA disrupted → Growth slows

What to watch for

Diarrhoea, mouth sores and skin soreness may become serious. Chest pain during the pump infusion needs emergency assessment. Unexpected severe early toxicity can occur even after dihydropyrimidine dehydrogenase (a medicine-processing enzyme) (DPD) testing.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Your indication and treatment aim

The same medicines can have different aims.

After colon-cancer surgery

Adjuvant treatment aims to reduce recurrence risk after complete resection. The likely absolute benefit depends on stage, pathology, fitness and other features. Ask how recurrence risk and neuropathy risk influence the proposed duration.

Advanced colorectal or upper-gastrointestinal (GI) cancer

Treatment may aim to control growth, relieve symptoms and prolong life. The plan is reviewed against response, toxicity and your priorities. A treatment break or a change of components requires an individual discussion.

How long will treatment last?

Do not infer duration from the number of passport pages. Ask for the planned number of cycles and review points. The NCCP schedule distinguishes adjuvant colon treatment, metastatic colorectal treatment and upper-GI treatment.

Understanding the evidence

Ask which evidence matches your cancer, treatment aim and medicines.

Adding oxaliplatin to fluorouracil-based treatment improved long-term outcomes in a study of people with stage III colon cancer after surgery. That study used FOLFOX4, a related but different schedule. It does not predict benefit for every indication of modified FOLFOX6 or for an individual person.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

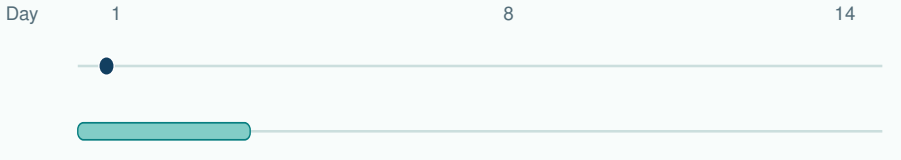
Your treatment stages, checks and practical preparations, from before treatment through follow-up.

Modified FOLFOX-6
NCCP 00209

Your treatment plan

Modified FOLFOX-6

14-day view



This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

INFUSION + HOME PUMP

14-day treatment cycle

D1 D2 D3 D14

Day 1: oxaliplatin, folinic acid and fluorouracil, followed by a 46-hour fluorouracil pump. A cycle lasts 14 days.

The treating team confirms the indication, schedule and duration. Changes are recorded on your own prescription.

BASELINE → TREATMENT → FOLLOW-UP

Checks across the whole treatment course

The columns show the order of care, not equal lengths of time. Confirm who books each check, who reviews it and whom to call if it is missing.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00209 v10a · official regimen

Tests in your plan

Carcinoembryonic antigen (CEA) — colorectal plans

If part of your plan

WHEN

For a colorectal cancer plan, if your team uses CEA; your team sets the interval.

A marker trend interpreted alongside symptoms, examination and scans.

Record the baseline, latest result and next date. A marker change alone does not establish progression.

THE WHOLE TREATMENT JOURNEY

Modified FOLFOX-6: treatment and checks

Gastrointestinal cancers

Day 1: oxaliplatin, folinic acid and fluorouracil, followed by a 46-hour fluorouracil pump. A cycle lasts 14 days.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Each treatment cycle	Between doses	Decision / response review	Completion / change
Blood count / kidneys / liver NCCP 00209	Baseline bloods; DPD before fluoropyrimidine	Repeat before each cycle	Earlier tests if unwell, dehydrated or toxicity develops	Results and recovery guide any delay or dose change	Follow unresolved abnormalities
Symptoms and function NCCP 00209	Document existing symptoms and usual activity	Review worst symptoms since the last treatment	Use urgent contacts; do not wait for the next visit	Balance benefit, side effects and priorities	Agree recovery care and ongoing support
Neuropathy / cold sensitivity NCCP 00209	Record existing nerve symptoms	Check persistence and effect on daily tasks	Report worsening numbness or cold-triggered throat symptoms	Consultant reviews the oxaliplatin component separately	Nerve symptoms can continue after treatment; arrange review
Heart checks NCCP 00209	Review cardiac history; electrocardiogram (ECG) when cardiac function is compromised or clinical findings warrant it. No automatic serial echocardiogram schedule is specified for this regimen.	Ask about new chest pain or breathlessness	Chest pain needs urgent assessment	Repeat heart tests if clinically indicated	Follow any heart problem; no fixed echo series

Care / check	Before starting	Each treatment cycle	Between doses	Decision / response review	Completion / change
Response / scans NCCP 00209	Confirm cancer setting, disease assessment and intended duration	Review clinical benefit and symptoms	New symptoms may bring review forward	Consultant-directed scans and response assessment	Agree follow-up, completion or another treatment

What each check is for

1 **Confirm your personal plan**

Before starting

Modified FOLFOX-6 combines oxaliplatin, folinic acid and fluorouracil. It can be used after colon-cancer surgery or to treat advanced colorectal, stomach or gastro-oesophageal-junction cancer. The aim and duration depend on the indication; ask which applies to you.

Ask / act: Agree medicines, treatment days, duration and monitoring.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Your port and treatment pump

From preparing the line to coming home after disconnection. Your team arranges the appointments and shows you what to do.

1 · Before treatment

Choose and prepare the line

The team explains whether a port or peripherally inserted central catheter (PICC) suits your treatment. You receive instructions for the insertion appointment.

2 · Insertion and recovery

A separate appointment may be needed

For a port, numbing medicine is used and a small device is placed under the skin. Ask about sedation, transport and wound care.

3 · Day 1

The nurse connects your pump

After the day-ward medicines, the nurse checks and connects the fluorouracil pump. Ask for a demonstration before leaving.

4 · At home

Treatment continues for about 46 hours

Carry the pump as shown. Keep the dressing dry and protect the line from pulling, heat and damage.

5 · As arranged

Return for disconnection

A trained professional checks the infusion, disconnects the pump and cares for the line. The appointment is based on connection time.

6 · Between cycles

The access device usually stays

A port needle can be removed while the port stays under the skin. A PICC remains in the arm with ongoing dressing and line care.

Port or PICC: what is the difference?

A port is a small chamber beneath the skin, usually on the chest, attached to a tube ending in a large vein. A special needle goes through the skin into the port when it is used. A PICC enters a vein in the upper arm; a short section remains outside under a dressing. Both can deliver a home infusion. Your team helps choose the device and teaches its care.

Before insertion and during recovery

Tell the team about allergies, blood thinners, infections and any other cancer medicines. Follow the instructions about eating, drinking and transport; do not stop medicines yourself. Port placement uses local anaesthetic, sometimes with sedation or general anaesthetic. The team explains the procedure and risks, including infection, bleeding, clots and insertion complications. Ask how to manage discomfort, protect the wound and resume activity. Report increasing pain, redness, swelling, discharge or fever.

Connection: what the nurse does

The nurse checks the line and treatment, secures the connection and dressing, and starts the pump. Before leaving, you should have the disconnection location and time, contact numbers including out of hours, and the written spill instructions. Ask to be shown what normal pump progress looks like. You do not need to calculate a rate.

Carrying it, sleeping and washing

Use the supplied carrier and the position recommended for your pump. Keep tubing free from pulling and kinks. Ask how to place the pump safely for sleep and how to protect the line and dressing when washing. Do not immerse it or swim. Avoid direct heat and cold. An elastomeric pump has a balloon that shrinks slowly and may have no alarm; an electronic pump has different instructions. Follow the instructions for the device you actually receive.

Disconnection is not port removal

Keep the appointment even if the pump looks empty, unless the team changes the plan. Staff check the pump and line and arrange safe disposal. After flushing and other line care as required, the port needle may be removed and a small dressing applied. The implanted port stays in place until a separate removal procedure is arranged. A PICC remains in place and needs scheduled dressing and flushing care.

Leakage, disconnection or unexpected pump progress

Call the team immediately if the pump leaks, is damaged, finishes unexpectedly early, appears not to run, or the line disconnects. Follow the emergency or spill steps you have been specifically taught, including use of a clamp if instructed. Do not reconnect, change settings or disconnect the pump yourself. Keep other people away from leaked medicine. If medicine touches skin, rinse it off and wash with soap and water, then seek advice.

Pain, swelling or feeling unwell

Contact oncology immediately for pain or swelling around the line, a wet/loose dressing, redness, fever, shivering or suddenly feeling unwell. A painful swollen arm or neck needs urgent advice. Severe chest pain, major breathing difficulty or collapse: call 112 / 999. Do not wait for pump disconnection or the next clinic visit.

The drawings show general steps, not instructions for inserting or accessing a line. Your hospital confirms its own arrangements and the care instructions for your device.

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Before the first treatment

Confirm the aim, likely course length, blood-test arrangements and a working urgent contact number. DPD testing is required before starting the fluoropyrimidine unless already completed.

STEP 2

Line and pump arrangements

Confirm venous access, the 46-hour fluorouracil pump and disconnection arrangements before leaving. Know whom to contact for leakage, an alarm, line pain or a pump that finishes unexpectedly.

STEP 3

If treatment is delayed

Ask the unit to confirm the new treatment and blood-test dates. Re-anchor supportive medicines to the treatment actually given; do not take an extra or catch-up anticancer dose yourself.

STEP 4

Getting help between visits

Follow your oncology unit’s urgent advice for fever, infection, severe diarrhoea, chest pain or difficulty breathing. Bring your treatment card and current medicines list.

What the checks are for

Symptoms, blood results and the treatment aim answer different questions.

Blood counts

Neutrophils protect against infection, haemoglobin carries oxygen and platelets help clotting. A normal previous result or feeling well does not prove the current counts are safe. Fever or sudden illness needs immediate advice.

Kidney and liver function

Organ function can change drug exposure. Vomiting or diarrhoea can cause dehydration and worsen kidney function. Report reduced urine, poor intake, jaundice and new medicines. The clinical assessment combines results with symptoms.

Describe symptoms in practical terms

Tell the team about the worst symptoms since the last visit, even if improved today. Examples include extra stools above your usual pattern, whether mouth pain affects drinking, and whether numbness affects buttons or balance.

How benefit is assessed

After surgery, treatment aims to reduce the chance of recurrence; there may be no visible cancer to measure. In advanced disease, scans, symptoms and function help assess control. Ask when review is planned and what would lead to a change.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Modified FOLFOX-6
NCCP 00209

Day 1 → Day 14: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
Day-ward medicines	 **Day 1** intravenous (IV)
Fluorouracil pump	 **Days 1–3** 46 hours; disconnect as arranged
Cold sensitivity	 **Days 1–5** Often early; may last longer
Blood counts / infection	 **Days 1–14** Timing varies; no guaranteed safe days
Mouth / bowel / energy	 **Days 1–14** Record symptoms throughout

Treatment and pump: Day 1 treatment is followed by a 46-hour fluorouracil infusion. The team confirms the exact pump disconnection time.

Cold sensitivity: Tingling or throat discomfort can occur early and may persist; follow the precautions your team gives you.

Blood counts: Counts can fall between visits. The day of the cycle and how well you feel cannot rule out infection.

Symptoms over time: Mouth and bowel symptoms or fatigue can overlap. Numbness may accumulate across cycles; describe changes before the next dose.

Why these bands stay broad: Bars show planned treatment and possible symptom patterns, not measured risk. Side effects can overlap and persist. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00209 v10a · official regimen ↗ · eviQ: patient information (supportive explanation; NCCP schedule takes priority) ↗

There are no guaranteed "safe days". Act on fever, sudden illness or a new serious symptom whenever it occurs. Do not wait for the next appointment.

Between-treatment notes

Day 1: Infusion and pump connection

Symptoms and blood tests are reviewed. The day-ward medicines are followed by connection of a fluorouracil pump. Tell the nurse immediately about a reaction or pain at the infusion site. Before leaving, confirm the pump-care instructions, disconnection appointment and contact number.

Day 2–3: The pump is delivering treatment

The infusion runs for about 46 hours from connection; the calendar day does not determine the exact finish time. Follow the instructions for keeping the line dry and secure. Avoid cold exposure if it triggers symptoms. Call for a leaking, disconnected or unexpectedly stopped pump. Do not reconnect, alter settings or disconnect it yourself unless specifically trained and instructed.

Day 4–7: After disconnection

Nausea, diarrhoea, mouth symptoms, fatigue or cold sensitivity may continue. Use prescribed support. New symptoms deserve advice even if the pump has finished. Report poor intake, persistent diarrhoea or mouth pain early. Chest pain or major breathing difficulty requires emergency help.

Day 8–14: Between visits and the next review

Blood counts can fall between visits. Numbness may accumulate over cycles. Describe any difficulty fastening buttons, walking or handling objects before more oxaliplatin. Bring the symptom passport and check the next appointment. Day 1 usually repeats 14 days after this cycle started, only if confirmed.

Support around the medicines

1 **Nausea prevention and rescue**

Take the prescribed prevention medicines at the planned times and keep rescue treatment available. Tell the team if nausea or constipation is not controlled; do not duplicate steroid or anti-sickness medicines.

2 **Bowel symptoms**

Report diarrhoea promptly and follow the written rescue plan. Severe symptoms, fever, dehydration or inability to drink need urgent advice.

3 **Counts and recovery**

White-cell growth-factor injections are prescribed according to individual risk and previous complications. They are not automatically required for every patient or every cycle.

4 **Cold and nerve symptoms**

Avoid cold exposure as advised after oxaliplatin. Report persistent numbness or difficulty with buttons, walking or everyday tasks before the next dose.

5 **No ice chips with oxaliplatin**

Do not suck ice chips during bolus fluorouracil in this regimen: oxaliplatin can cause cold-triggered nerve symptoms. New breathing difficulty still needs clinical assessment.

Living with the 46-hour pump

Ask for a demonstration and written contact instructions.

Before leaving

Know what normal pump progress looks like, how to carry it and how to protect the line while washing or sleeping. Check who will disconnect it and when. Have the team check anything you do not understand.

Leakage or a delivery problem

Use the hospital spill instructions, avoid direct contact with leaked medicine and call the team. Do not repair a connection or adjust the pump. A pump finishing much sooner or later than expected also needs advice.

Chest pain while on fluorouracil

Fluorouracil can affect the heart. New chest pain during treatment needs urgent assessment; severe pain, collapse or major breathing difficulty means 112/999. Tell emergency staff that fluorouracil is infusing. Follow the team's instructions about the pump.

Recognising treatment effects

Early reporting helps the team manage symptoms and protect function.

Acute cold sensitivity and persistent neuropathy

Cold-related tingling can occur soon after oxaliplatin. Persistent numbness between cycles is different and may worsen with repeated treatment. Describe duration and effect on hand use or walking. Some neuropathy can remain after treatment ends.

Bowel and mouth symptoms

Fluorouracil can affect rapidly renewing tissues. Keep track of stools compared with your normal pattern, including stoma output. Report diarrhoea, painful ulcers or reduced drinking; do not wait for the next cycle if you are dehydrated or unwell.

Hand-foot soreness and skin changes

Tell the team about red, tender or peeling palms or soles. Reduce friction and use the skin care recommended locally. Pain limiting normal activity needs review. Do not assume it must be tolerated to complete treatment.

Fatigue and nausea

Use manageable activity, rest and the prescribed anti-sickness plan. New profound weakness, faintness or breathlessness warrants assessment because anaemia, infection, dehydration and other problems can look like ordinary fatigue.

Living with treatment

Plan around the activities and people that matter to you.

Food, drink and practical help

Use foods and drinks you can manage. Tell the team about weight loss, poor intake or difficulty shopping and cooking; ask for dietitian or practical support. A fixed daily fluid target is not appropriate for everyone, particularly with heart or kidney problems.

Activity, rest and driving

Balance rest with manageable activity. Record what fatigue stops you doing. Do not drive when dizzy, unwell or affected by sedating medicines. Ask about work adjustments and transport for treatment days.



Medicines, vaccines and supplements

Bring a complete medicine list, including over-the-counter products and supplements. Ask the oncology pharmacist before changes. Discuss vaccination timing. Do not stop prescribed long-term medicines on your own.

Fertility, contraception and breastfeeding

Discuss fertility preservation before treatment when relevant. Ask for a written contraception and breastfeeding plan covering each medicine and the period after treatment. Tell the team promptly about a possible pregnancy.

Support for you and your family

Ask about psycho-oncology, social work, counselling and symptom support. Take information in manageable amounts. A family member or trusted person can help record questions if you wish. Discuss how treatment fits your priorities.

Words you may hear

A short reference for clinic discussions.

Adjuvant, metastatic and cycle

Adjuvant means treatment after the main treatment, usually surgery, to reduce recurrence risk. Metastatic means spread to a distant site. A cycle includes treatment and the interval before the next planned start.

DPD, neuropathy and dose modification

DPD is an enzyme involved in fluorouracil breakdown. Neuropathy means nerve damage or dysfunction. Dose modification means changing the amount, timing or components to balance benefit and harm.

Questions worth asking

Does having more side effects mean treatment is working?

No. Side effects do not reliably show whether the cancer is responding. Report them so they can be treated. Your team assesses benefit using the tests and review plan appropriate to your cancer.

Why might a dose be delayed or reduced?

Your team may allow recovery, investigate a symptom or reduce the chance of lasting harm. Ask which drug changes, why, and when review is planned. A delay does not itself mean treatment has failed.

What should I say when calling?

Give the regimen name, last treatment date, symptoms and when they started. Explain whether you can eat, drink, pass urine and manage usual activities. Give your temperature if available, but do not delay calling to collect details.

What belongs in the consent conversation?

The individual aim and likely benefit, uncertainty, reasonable alternatives including not proceeding, common and serious risks, lasting effects, fertility and practical concerns. You can ask for an interpreter, more time or another explanation and can reconsider your decision.

How many cycles should I use this passport for?

The passport provides reusable recording pages, not a prescribed course length. Your oncology team sets the duration and can provide extra pages.

Can I use this page to decide if blood results are safe?

No. Treatment decisions require the clinical protocol, symptoms, previous toxicity, organ function and the prescriber's review. The patient page explains the checks and does not authorise treatment.

Try explaining the plan back: Can you explain which medicines you are receiving, the schedule, the contact plan and the symptoms that need prompt help?

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Modified FOLFOX-6 · active treatment cycle

Active medicines: Oxaliplatin + Folinic acid + Fluorouracil

Emetogenic risk: Moderate

Medicine	Dose / route	When
Ondansetron	16 mg orally once	Before Day 1 chemotherapy Review QT risk, interactions, constipation and hepatic function.
Dexamethasone	8 mg orally once	Before Day 1 chemotherapy Reconcile all steroid prescriptions; do not duplicate.
Dexamethasone	8 mg orally once daily	Days 2 and 3 after Day 1 chemotherapy Confirm the prescribed delayed prophylaxis and re-anchor it if treatment is delayed.
Metoclopramide	10 mg orally up to three times daily as required	Rescue only; duration to be completed by prescriber Review renal function, neurological contraindications and interactions.

Additional phase-specific support

- Take the prescribed prevention medicines at the planned times and keep rescue treatment available. Tell the team if nausea or constipation is not controlled; do not duplicate steroid or anti-sickness medicines.
- Report diarrhoea promptly and follow the written rescue plan. Severe symptoms, fever, dehydration or inability to drink need urgent advice.
- White-cell growth-factor injections are prescribed according to individual risk and previous complications. They are not automatically required for every patient or every cycle.
- Avoid cold exposure as advised after oxaliplatin. Report persistent numbness or difficulty with buttons, walking or everyday tasks before the next dose.
- Do not suck ice chips during bolus fluorouracil in this regimen: oxaliplatin can cause cold-triggered nerve symptoms. New breathing difficulty still needs clinical assessment.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

What to watch for

You do not need to know which medicine caused a symptom before calling.

1 Mouth / eating

Fluorouracil

Report painful ulcers or difficulty eating and drinking.

2 Chest / heart

Fluorouracil

New chest pain needs urgent assessment, including during an infusion.

3 Bowel / hydration

Fluorouracil

Report diarrhoea, increased stoma output, severe pain, blood or dehydration.

4 Hands / feet

Fluorouracil

Report painful redness, cracking or peeling that affects daily activity.

5 Blood / infection

Chemotherapy

Fever, shivers or sudden illness need immediate oncology advice.

6 Energy / daily life

Treatment

Report fatigue that limits usual activity; ask for practical and nutrition support.

7 Nerves

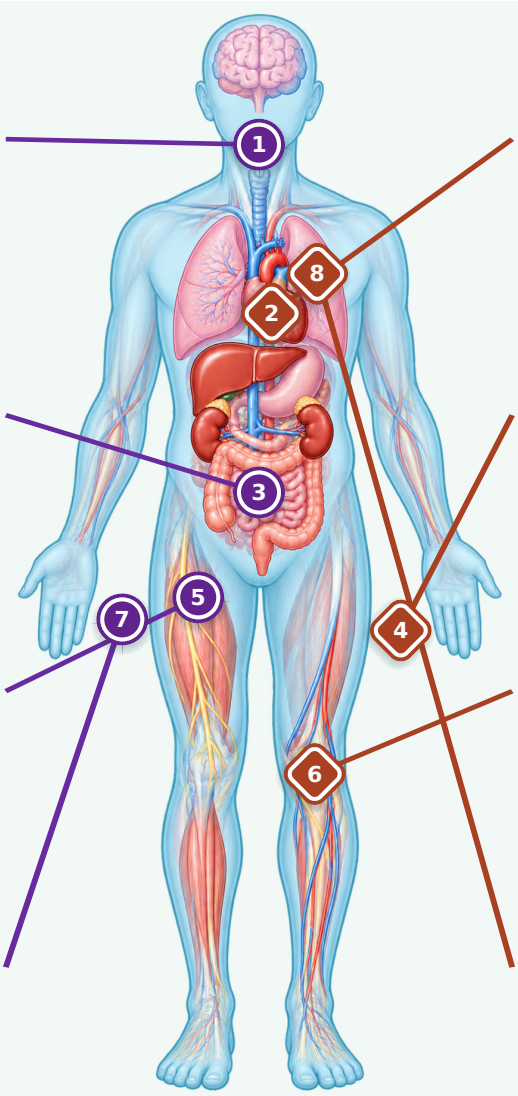
Oxaliplatin

Cold can trigger tingling. Report lasting numbness or difficulty with buttons or walking.

8 Throat / breathing

Oxaliplatin

Report throat tightness or breathing difficulty promptly. Tell staff immediately about infusion-site pain or swelling.



Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

New chest pain, breathing difficulty, severe diarrhoea, inability to drink, reduced urine, uncontrolled bleeding or a rapidly worsening symptom needs prompt assessment.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

CHAPTER 04

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **National Cancer Control Programme (NCCP) 00209 v10a · official regimen**
   https://healthservice.hse.ie/documents/6549/209_v10a_FOLFOX_6_Modified.pdf
2. **eviQ: patient information (supportive explanation; NCCP schedule takes priority) ↗**
   https://www.eviq.org.au/medical-oncology/colorectal/metastatic/114-colorectal-metastatic-folfox6-modified-fluo/patient-information
3. **eviQ: treatment through an implanted port**
   https://www.eviq.org.au/p/507
4. **eviQ: treatment through a peripherally inserted central catheter (PICC)**
   https://www.eviq.org.au/p/543
5. **eviQ: elastomeric infusion pump care**
   https://www.eviq.org.au/p/502
6. **HSE: chemotherapy side effects and urgent advice**
   https://www2.hse.ie/conditions/chemotherapy/side-effects/
7. **NCI: nausea and vomiting**
   https://www.cancer.gov/about-cancer/treatment/side-effects/nausea-vomiting
8. **MOSAIC: ten-year follow-up (2015)**
   https://pubmed.ncbi.nlm.nih.gov/26527776/
9. **NCI: colon cancer treatment — carcinoembryonic antigen (CEA) and follow-up**
   https://www.cancer.gov/types/colorectal/hp/colon-treatment-pdq
10. **NCCP NCIS antiemetics V8, Table 1; apply oral regimen guidance separately**
    https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf